

Direct Access (non-urgent) Upper GI Endoscopy Request Form

PLEASE EMAIL THIS FORM TO:

uhdb.bhft.burtonreferrals@nhs.net

Patient name	Given Name Surname		
Address	Home Full Address (single line)		
DOB	Date of Birth	NHS No.	NHS Number
Home tel. No	Patient Home Telephone	Gender	Gender(full)
Mobile Tel. No	Patient Mobile Telephone	Ethnicity	Ethnic Origin
Preferred Tel. No		E-mail address	Patient E-mail Address
Main spoken Language	Main Language	Able to communicate effectively in English or interpreter needed?	Yes ☐ No ☐
Transport needed	Yes ☐ No ☐	Contactable by telephone?	Yes ☐ No ☐

Practice name	Organisation Name		
Registered GP	Registered GP Full Name	Usual GP	Usual GP Full Name
Registered GP address	Organisation Full Address (single line)		
Tel. No	Organisation Telephone Number		
e-mail	Organisation E-mail Address	Practice code	Organisation National Practice Code

WHO performance status: (please tick for ALL patients)	
0 - Able to carry out all normal activity without restriction	☐
1-Restricted in physically strenuous activity but able to walk and do light work	☐
2-Able to walk, capable of all self-care. Unable to carry out any work. Up & about 50% of waking hours	☐
3-Capable of only limited self-care, confined to bed or chair more than 50% of waking hours	☐
4-Completely disabled. Cannot carry out any self-care. Totally confined to bed or chair	☐

Prior to referring the patient, please confirm	
Is the patient fit for a day-case gastroscopy	☐
Is the patient able to be observed at home overnight	☐
Is the patient on anticoagulation	☐
Is the patient on an antiplatelet medication	☐
Has the patient had a previous failed gastroscopy or gastroscopy within the last	☐

2years	
Meets the NICE (NG12): Suspected cancer: recognition and referral criteria for nonurgent referral for upper GI endoscopy:	☐
1. Haematemesis 2. Treatment resistant dyspepsia 3. Upper abdominal pain with low haemoglobin levels	
4. Raised platelet count with any of the following: ○ Nausea ○ Vomiting ○ Weight loss ○ Reflux ○ Dyspepsia 5. Upper abdominal pain, or 6. Nause or vomiting with any of the following: ○ Weight loss ○ Reflux ○ Dyspepsia ○ Upper abdominal pain 7. Patients with suspected coeliac disease (and non-diagnostic serology, tTG < 50) can be referred for direct access gastroscopy and biopsy. All patients require a gluten containing diet for at least 4 weeks prior to endoscopy.	
Edinburgh Dysphagia Score <3.5 (see below)	☐
Please confirm that you have indicated the WHO functional status	☐
Does the patient have the cognitive capacity to discuss / consent to gastroscopy	☐

Edinburgh dysphagia score (EDS):

A Age	Points	B Gender	Points	C Current acid reflux	Points	D Dysphagia localises to neck	Points
18-39	0	Male	0	Yes	-1	Yes	-2
40-49	4	Female	-1	No	0	No	0
50-59	5						
60-69	6						
70-79	7						
80-89	8						
90-99	9						
EDS= A+B+C+D+E+F							

If score ≥3.5 patient requires urgent investigation as higher risk of cancer

If score <3.5 patient can have routine endoscopy

In the event of significant findings requiring urgent attention, I agree to my patient being referred and dealt with as appropriate.

Additional Information:

Signature:		Name of Referring	
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		GP:	
Date:	Short date letter merged		

Problems
Medication
Allergies

NB: Referrals will only be accepted via the below email

uhdb.bhft.burtonreferrals@nhs.net